



Transforming American Healthcare

A Six-Pillar Framework for
System Transformation



Policy
(The mandatory architecture)

Economics
(The incentive architecture)

Technology
(The data substrate)

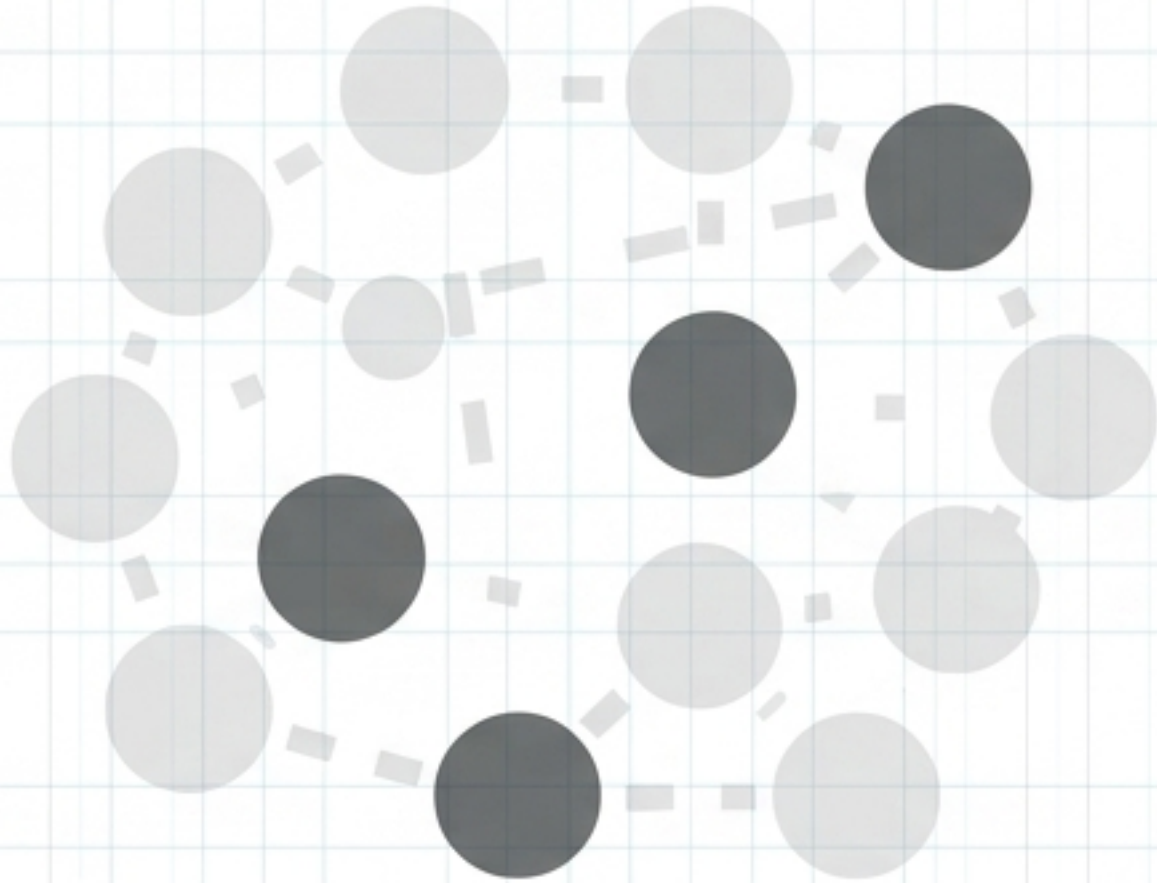
Clinical
(The mechanism of change)

Equity
(The cross-cutting constraint)

Operations
(The execution layer)

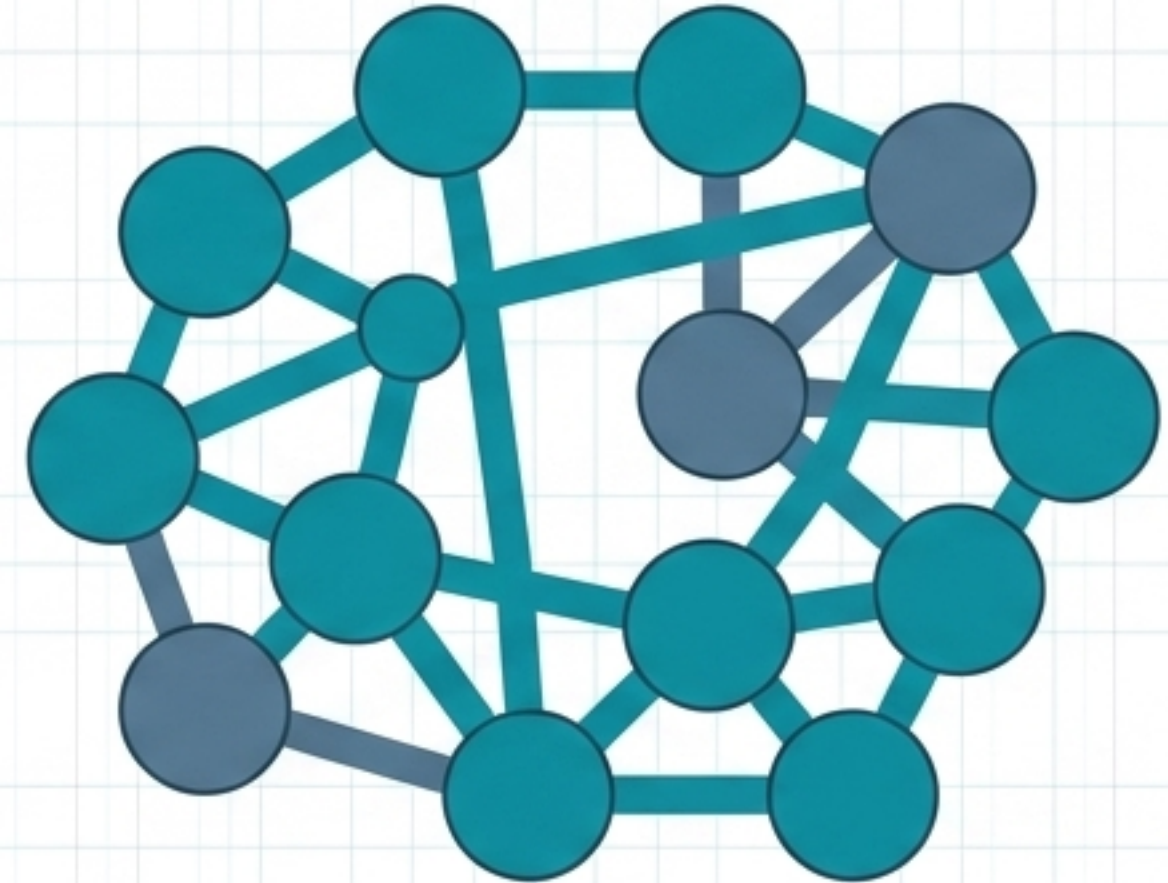
		Source							
		Policy	Economics	Technology	Clinical	Equity	Operations		
Target	Policy	∅ Null	✓	∅ Null	∅ Null	✓	✓	Key Highlights Policy → Economics: Enables mandatory payment reform. Economics → Clinical: Drives care delivery redesign. Technology → All 5 Pillars: Enables analytical management. Operations → Policy & Technology: Closes the feedback loop.	
	Economics	∅ Null	∅ Null	∅ Null	✓	∅ Null	✓		
	Technology	✓	✓	✓	✓	✓	✓		
	Clinical	∅ Null	∅ Null	∅ Null	∅ Null	✓	∅ Null		
	Equity	∅ Null	∅ Null	∅ Null	∅ Null	∅ Null	✓		
	Operations	✓	∅ Null	✓	∅ Null	∅ Null	∅ Null		

The Voluntary Era



- High-cost actors opt out
- Partial participation
- Fragmented data

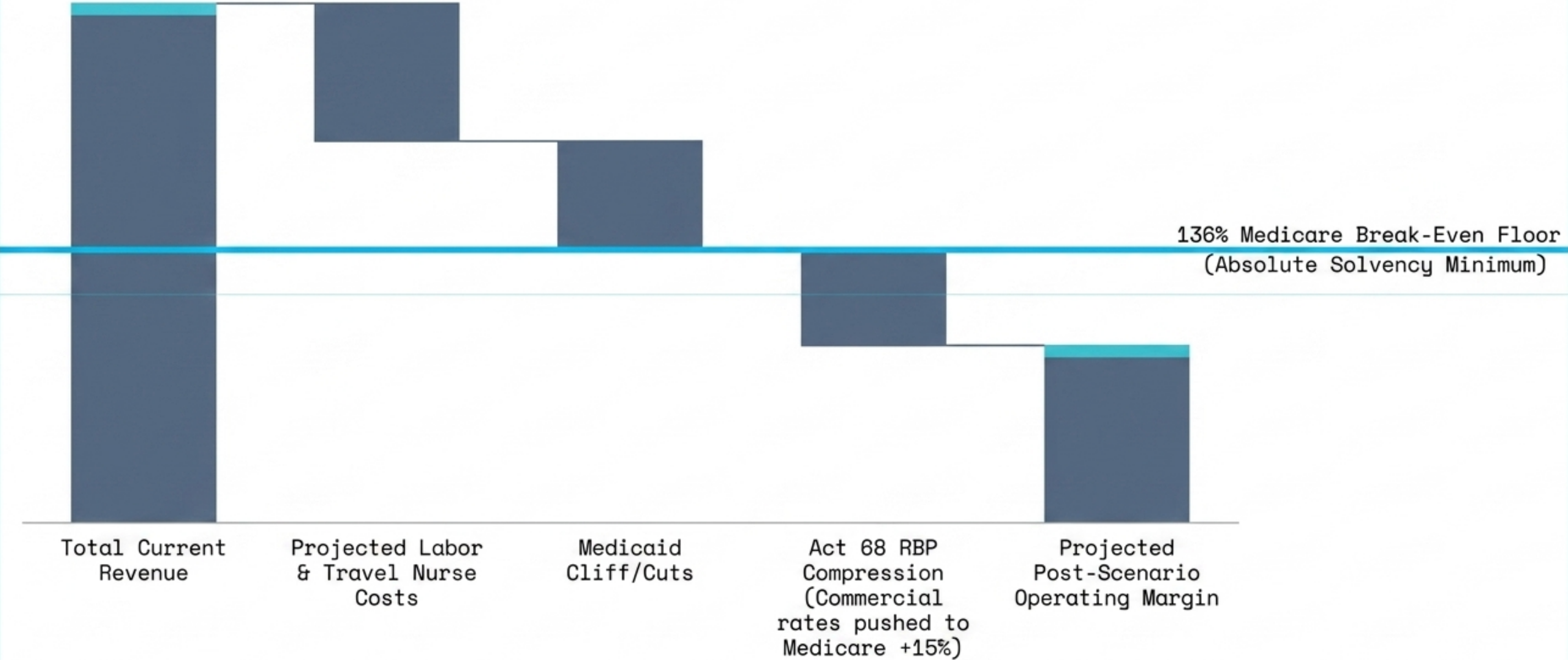
The Mandatory Architecture (Acts 167 & 68)



- Eliminates voluntary opt-out
- Statutory deadlines force execution capacity
- Embeds equity accountability into law

The AHEAD Model (9-year horizon) provides the federal vehicle for state-level transformation.

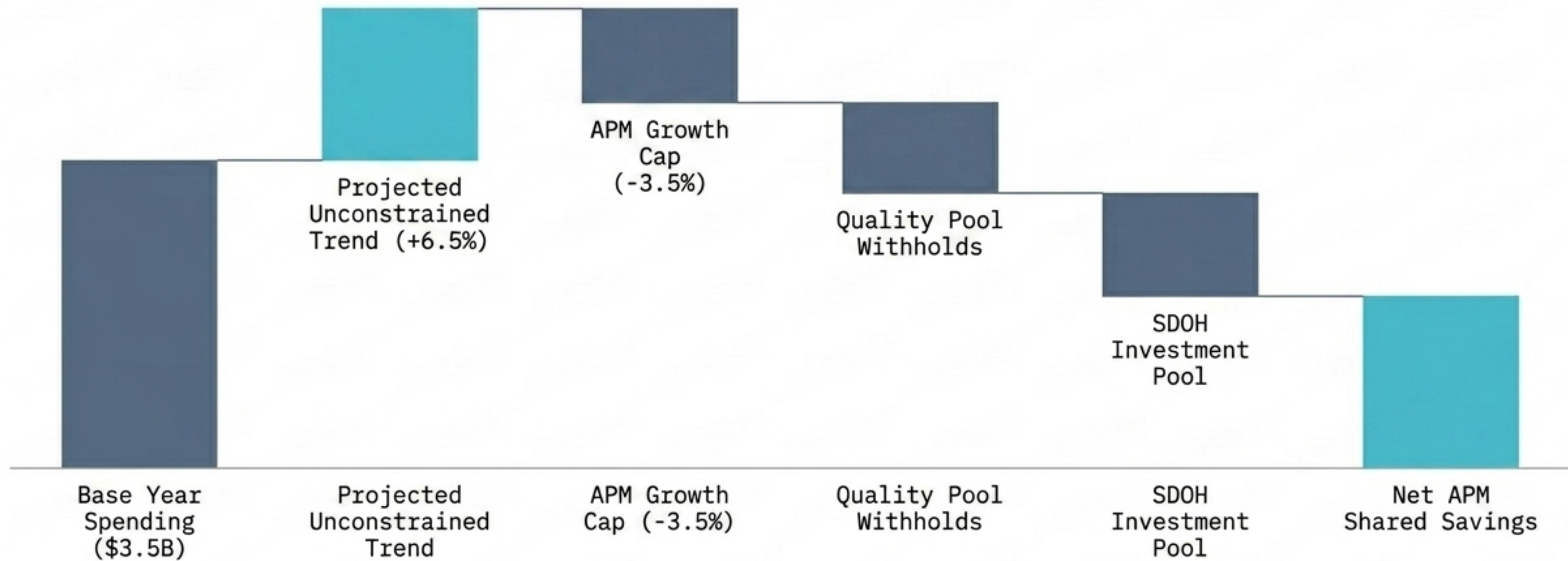
Hospital Financial Solvency Stress Test

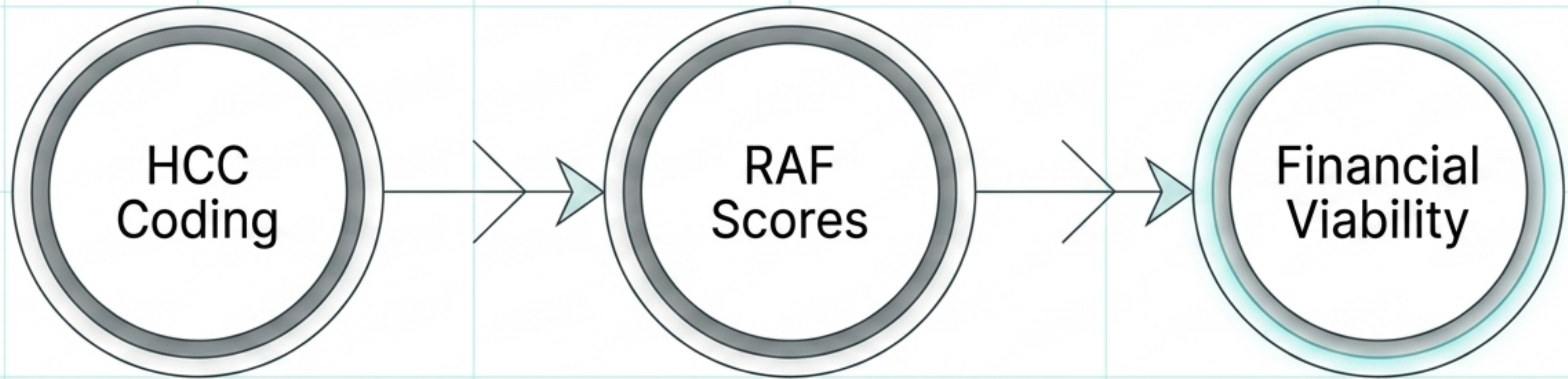


Value-Based Care Shared Savings Generation

Core Formula Callout

$$\text{annualBenchmark} = \text{attributedLives} \times \text{benchmarkPMPM} \times 12$$





The Operational Reality

Global budgets require pinpoint accuracy in population risk assessment.

RAF Gap Formula Application

$$\text{starRatingImpact} = \text{membershipSize} \times (\text{starBonusPct} \times \text{revenuePMPM}) \times 12$$

Execution Metrics

- HCC capture rate $\geq 95\%$
- Potentially Avoidable Utilization (PAU) tracking
- Regionalization: Assigning 14 hospitals into distinct Service Tier Nodes (COE, Rural, CAH)

Redesigning the Care Model

From episodic volume to continuous panel management.

Diabetes

Intensive Mgmt Program

Efficacy: 0.42 | Cost/Person: \$1,800

Heart
Failure

Hospital at Home

Efficacy: 0.35 | Cost/Person: \$3,200

COPD

Rehab/CCM Bundle

Efficacy: 0.45 | Cost/Person: \$1,500

Interventions must generate savings greater than their cost to maintain the global budget cap.

The Mathematical Imperative

The Trap:

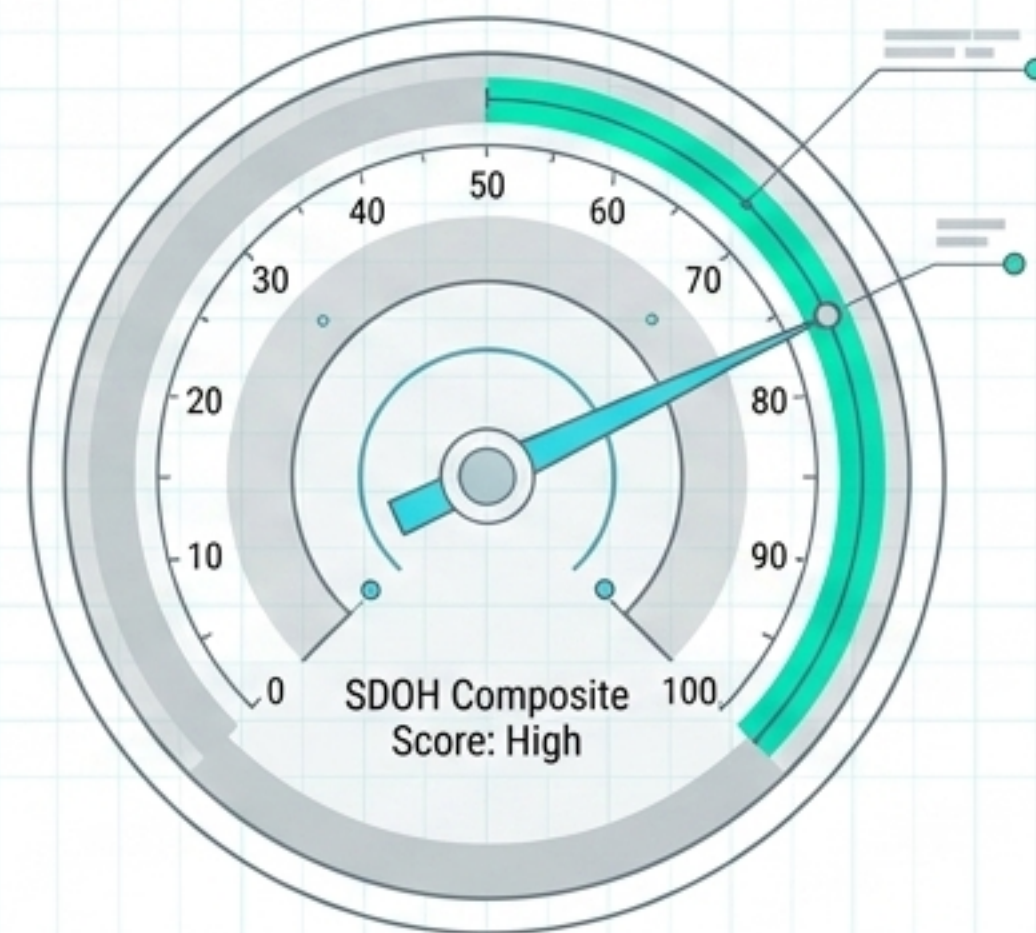
Transformation that improves averages while widening disparities has failed.

HEROI Framework Formulas

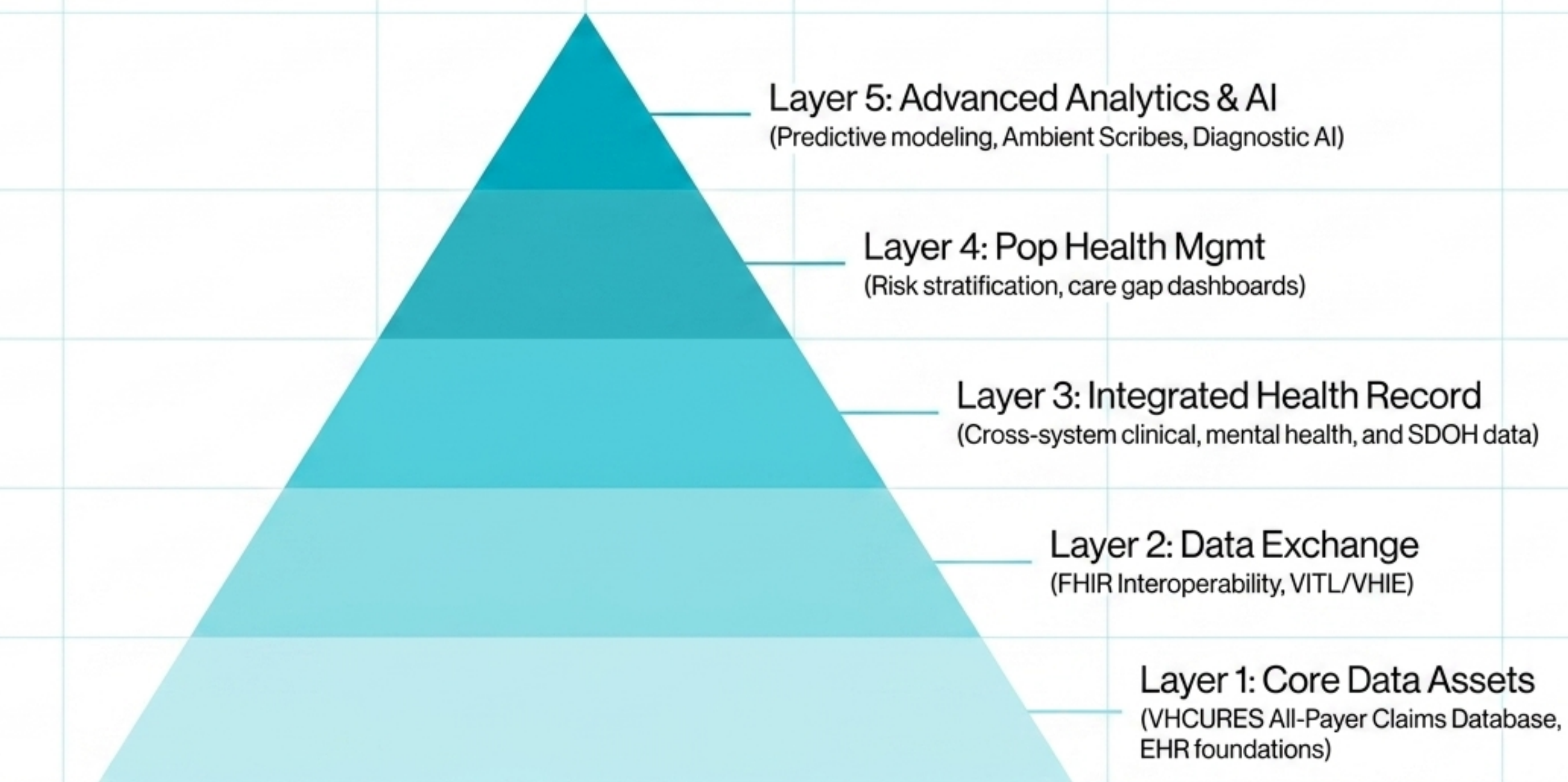
$$\text{disparityIndex} = \frac{(\text{weightedRate} - \text{allWhiteRate})}{\text{allWhiteRate}} \times 100$$

$$\text{lifeExpectancy} = 66.0 + (\text{compositeSDOH} / 100) \times 14.0$$

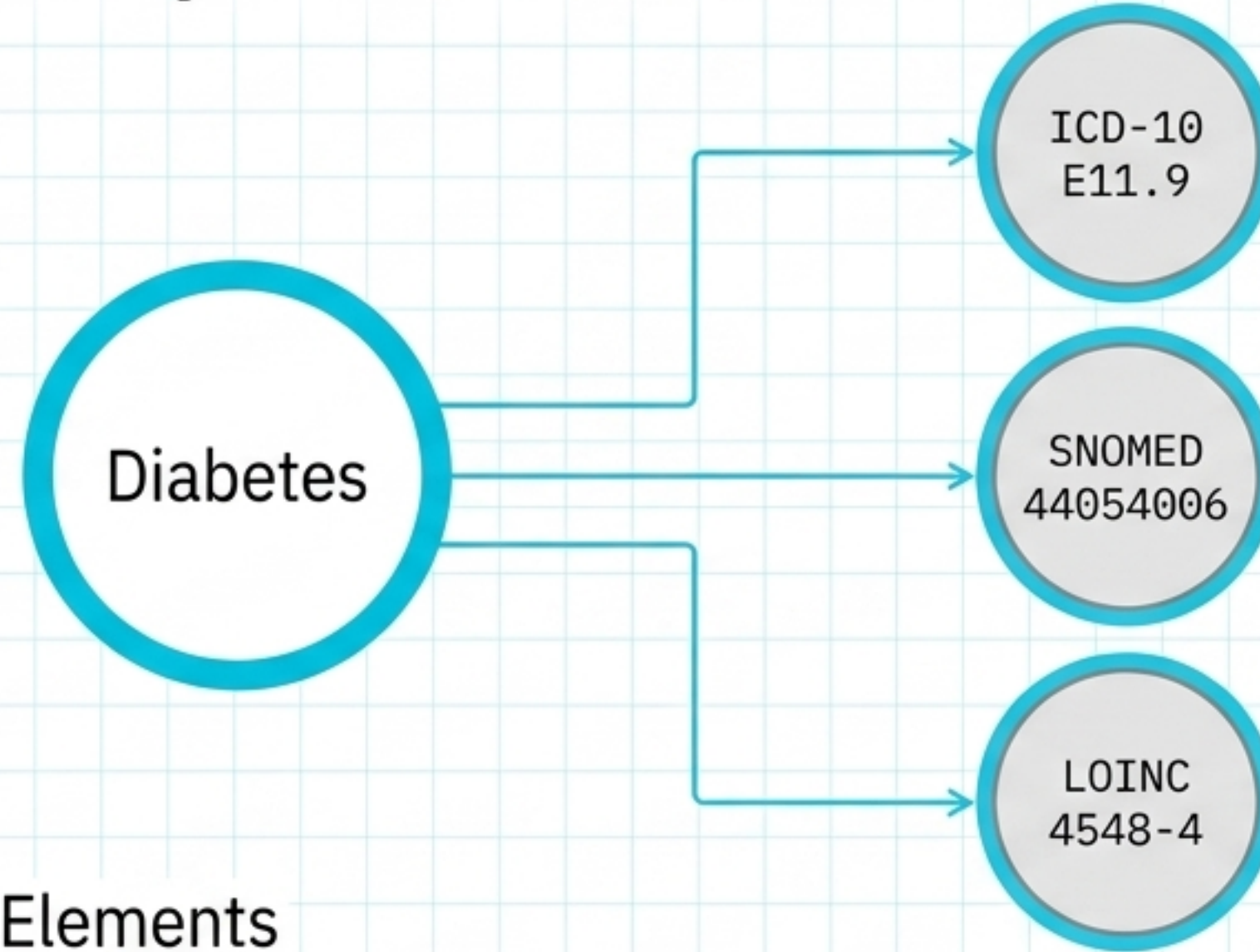
SDOH Measurement



Geographic access gap analyzers combined with stratified HEDIS measurement ensure investments reach the most deprived populations.



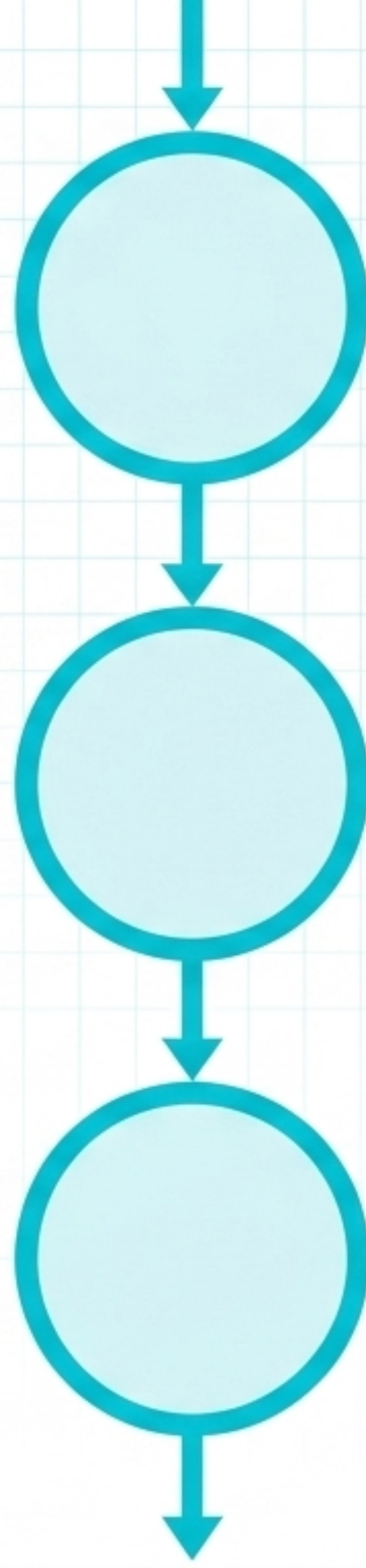
FHIR Interoperability in Practice



Technical Infrastructure Elements

- ⚙️ CDS Hooks Engine for real-time clinical decision support.
- 📄 Prior Authorization Automation via CRD (Coverage Requirements Discovery).

Outcome: Breaking the administrative bottleneck that consumes clinical workforce capacity.



Level 1: State Architect

Sets strategic direction, payment reform framework, capital allocation, and monitors system performance

Level 2: Regional Convener / HSA Coordinators

Cross-program integration at the community level

Level 3: Provider Operator

Direct delivery of Medicaid, mental health, and community services

Housing
(SDOH Integration)

Clinical Care
(Hospital & Primary
Care Network)

**HSA
Coordinator**
(Regional Convener)

AHS restructuring removes
programmatic silos, placing the
geographic community at the
center of resource allocation.

Transport
(Access Infrastructure)

The HTI Composite Dashboard

20%

Digital Maturity

20%

Social Determinants

20%

Clinical Excellence

15%

Value-Based Care

15%

Patient Experience

10%

Workforce Wellness

Transformation is not a narrative; it is a measurable, mathematically trackable reality.



Vermont as the National Preview. What Vermont solves today under Acts 167 & 68, the rest of the nation must solve tomorrow. Transformation requires all six pillars moving as one.